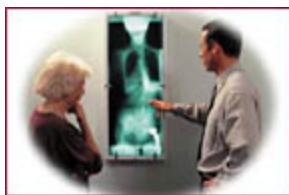


INSTITUTE FOR
CAREER RESEARCH

**CAREER AS AN
OSTEOPATHIC
PHYSICIAN
DOCTOR OF
OSTEOPATHY (DO)**





**YOU CAN HELP AND TREAT PATIENTS
IN THIS RECOGNIZED AND RESPECTED
HEALTHCARE FIELD**

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Overview

Earnings

- Medical residents' salaries: \$30,000 to \$40,000
- Physicians in medical specialties: \$130,000 to \$210,000

Traits Required

- Able to meet the very high academic & personal demands of the work
- Dedicated to serving the public
- Require considerable patience & composure
- Comfortable with people
- Good under pressure

The Work You Will Do

- Spinal manipulation
- Order series of tests to learn if pain is caused by disease
- Take patient histories
- Deliver babies

Where You Will Work

- Hospitals
- Office based practice
- Clinics
- Government Agencies

Education Required

- Four years of undergraduate work
- Four years of osteopathic college
- At the least, three years of residency

Pluses

- Many Opportunities
- Future outlook is good
- Excellent earnings

Minuses

- Long & irregular hours
- Competition
- Respect

Introduction

Donald, 28 years, has tried several fields in the course of his lifetime, professional tennis player, risk capital associate, and teacher, but none has panned out as something to build upon for the future, for various reasons.

So while glancing through a career manual in search of a career that would be more to his liking and in line with his talents, he came across the term DO, doctor of osteopathy. Like many Americans, Don had never before heard the term, and didn't know what osteopaths do. On further exploration, he discovered that osteopathy, or osteopathic medicine, is one of the two branches of medicine.

Like the MD (Medical Doctor) or "allopathic" branch of medicine, as it is sometimes known, osteopathic physicians, or DOs have the same kind of training, go to medical school for the same number of years, and can practice medicine in all of its specialties in all 50 states. What's more, although most DOs are in primary care -that is they are the first physicians that the patient is likely to come into contact with (pediatricians, internists, family practitioners and obstetricians) - they often go on to further training as specialists and can be certified in any of the various medical specialties, the same as Mds.

DOs can admit patients to hospitals, prescribe medications, or any of the various medical treatments such as physical or occupational therapy, and deliver babies. In other words, they are full-fledged physicians in every sense of the word.

At any rate, when Don mentioned that he was considering osteopathic medicine as well as allopathic medicine as a career, he asked his dad, a practicing physician, what he thought. Fortunately, his father was open minded and discovered that osteopaths were indeed licensed to practice medicine in all of its branches and in all states, the same as MDs. Eventually after several months of application to about eight schools including several osteopathic colleges, Don was accepted and he enrolled in one of the osteopathic schools, this one on the East Coast.

What decided Don on osteopathy more than anything else was its emphasis on the holistic approach - the relationships of the various body systems, digestion, nerves, blood, and other organs, to the patient's emotional and mental frame of mind and environment -where the patient worked, relationships with family, and other factors that entered into shaping personality and well being.

Osteopaths stress the interrelationship between all body systems as well as the musculoskeletal structure, and they believe that anything that can cause an imbalance in any one body system can cause changes in how the others function and may throw the body functions out of correct balance.

Don also learned about the DO's unique hands-on approach to spinal manipulation and palpation (touch), which are additional tools which these doctors use in diagnosing and treating disease.

DOs also emphasize the body's natural ability to heal itself and to stay healthy. And they believe it is imperative that the physician assist, support and otherwise help the body's own tendencies to maintain its health.

While exploring the various medical schools and osteopathic colleges, Don found out that the osteopathic schools seemed much more welcoming to older students who had been out of college for a few years. And in fact they seemed to go out of their way to seek him out, especially since his qualifications were excellent. He had scored very well on the MCAT (Medical College Admissions Test) and had nearly a 4.0 grade point average in his undergraduate college work, and good grades in biological science and in chemistry.

So even though both kinds of schools - medical and osteopathic - had their pluses, Don wound up selecting osteopathic school as his entree to the field.

Another who selected osteopathic college is 30-year-old Mary, mother of two, with an undergraduate degree in business and finance. What drew Mary to osteopathy was its emphasis on primary care, since serving patients directly and striving to work with patients in diagnosing and treating their various medical problems was her main interest in pursuing medicine.

Still, as Don and Mary found out, osteopathic medicine has its drawbacks. They found out that some hospitals in their community didn't know much about osteopathy, and doctors of the various hospital staffs were opposed to giving osteopathic physicians privileges in their hospitals.

And they found further that bias and ignorance of the medical profession often works against osteopaths being accepted into various specialty residency programs in areas which are heavily controlled by allopathic medicine. It might be very difficult to practice as a neurological surgeon or cardiovascular surgeon at certain prestigious hospitals.

While there still is a certain bias against accepting osteopathic graduates on the part of some medical residency training programs or in being admitted to the staffs of certain allopathic hospitals, relations between the two branches have never been better. Today, DOs in ever increasing numbers are being accepted on the staffs of hospitals -large and small - all over the country. They are also being accepted as members of various medical societies and increasingly are being approved for various residency specialty programs which in the past were restricted to graduates of allopathic medical schools.

There is no denying that the profession has made enormous strides in the past few decades. In 1967, for instance, there were only six osteopathic colleges in the United States; today there are 19 and the number is growing constantly. But it wasn't always that way. At one time, osteopathy maintained its own hospitals where DOs would have no trouble receiving staff positions and their own specialty certifying boards to accredit osteopathy's own training programs for the various medical specialties.

In just a few decades the profession has grown from 13,000 practitioners in 1969, to 30,000 in 1991 and almost 40,000 today. This huge increase in the number of practitioners is reflected in the number of students seeking admission to osteopathic colleges. This has been estimated at nearly 11,000 applicants in one recent year for the 2,200 slots available that year in the nation's 19 osteopathic colleges. This compared with over 45,000 students applying for admission to medical school of whom, about 16,000 were accepted.

So there's no doubt about it - the days when osteopathic medicine was often thought of as something to fall back on if you

were refused admission to allopathic medical school are definitely over. In fact, it is harder to get into osteopathic college today than ever before.

Although there are major similarities between the two branches of medicine, there are also several major differences. For one, osteopathic graduates are much more likely to go into primary medicine than their allopathic counterparts (two thirds as compared with one third) and unlike medical school graduates, many DOs are likely to practice in small-town rural communities, those of fewer than 50,000 population.

But the basic difference between the two branches is that DOs regard the body as a form and the various physiological systems as its functioning. To simplify matters, you might compare the body's musculoskeletal system - bones, muscles and nerves - to its container. And the various organs, blood and body systems as its inner workings. DOs believe the workings of the inner systems are very closely tied to the container, or the musculoskeletal system.

Get Involved Now

How do you know if you have what it takes to succeed in osteopathy? The answer is you don't - not until you have gotten some experience working either as a volunteer, or a paid employee in a hospital after school. Working in a hospital will bring you into some contact with patients and you will be able to better gauge your suitability for the career.

Beyond this, seek out some osteopathic physicians and see if you can talk to them. Find out what they like and dislike about the job -how much money you can realistically expect to make, how to get started, among other issues.

You should also be exploring the literature on the career available from professional organizations, to learn about what the work entails, requirements for admission and the suggested curriculum. Talk to as many students at osteopathic colleges as you can. They can be very helpful in answering questions on the osteopathic school program and just how tough it is.

Right now, in high school or college, you should be taking the courses that will help you to get started in this field. In high school take all the chemistry, biology and physics you can, as well as math. Since you will be dealing with the public in various degrees, you should be taking courses in public speaking and debate which will help you to feel at ease with people. Become active in Key Club, and other clubs in your school which help you to gain leadership and social skills, and be a leader in student government. In many cases, osteopathic school admissions directors look for such participation as an indication of your willingness to accept responsibility and leadership.

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History of the Profession

As one of the two major branches of medicine, osteopathic medicine goes back to ancient times. Almost since the beginning of recorded history, mankind has been concerned with problems of health and well being. One of the first such concerns is to be found in the Code of Hammurabi of Babylon, which dates back to 1727 BC. It states: "If a physician operates on a freeman and causes the man's death or blindness, the physician's hand will be cut off." The code further stated that if a slave's death resulted from a physician's treatment, the physician would have to pay for a replacement.

But in many ways, the foundation upon which medicine rests is to be found in ancient Greece, in the work of a physician named Hippocrates, to whom the highest standards of medical practice are attributed. It was Hippocrates who developed the code of medical ethics which physicians today still swear to uphold and which says in part: "I will practice my profession with conscience and dignity; the health of my patient will be my first consideration."

Hippocrates is also credited with developing the principles of the scientific method as he and his associates observed their patients. From these observations they drew certain conclusions as to the cause and treatment of the disease.

Leaping ahead over the centuries we come to the Holy Roman Emperor Frederick II, the first to recognize the connection of medicine to the welfare of mankind. In 1240 AD he wrote: "The

damage and suffering that occur to our subjects are due to the ignorance of physicians and must be remedied."

His solution: a series of laws requiring all physicians to obtain a diploma from a university followed by three years of study and a year's apprenticeship under an experienced physician. Surgeons were required to take a course in anatomy. This formed the prototype on which the study of medicine has been modeled over the centuries.

Later in the 17th century, the Italian scientist, Galileo, first used the compound microscope to examine the human body. But it was not until the pathfinding work of Edward Jenner, who in 1796 discovered the first smallpox vaccination which helped to bring this killer disease under control, that medicine really began to move forward. It has been estimated that in the past 200 years most of the major discoveries in medicine have been made. And since Jenner's historic work, the pace of medical progress and technology has really taken off. Today you can pick up a paper or view the news on TV almost every day and learn of new medical treatments and procedures in the cure of disease that were not even dreamed of less than a generation ago.

Jenner's historic work was followed by the discoveries of the brilliant French chemist, Louis Pasteur who discovered the process of using heat to control and eliminate the growth of germs in milk - the process which today is known as "pasteurization."

Working with Pasteur, the German scientist, Robert Koch, discovered specific organisms that cause disease. Known as the father of bacteriology, Koch was awarded the Nobel Prize in physiology and bacteriology in 1905.

Koch was followed by many other famous names in medicine: for example, Semmelweis, who identified the importance of antiseptic conditions in obstetrics and who diagnosed blood poisoning as the cause of disease which took the lives of many new mothers. There were many others: Morton, the father of the modern science of anesthesiology; Joseph Lister, an English physician, who demonstrated the importance of antisepsis, or germ control in eradicating disease; and Roentgen, the German physicist, who pioneered the concept of showing how a beam of electrical current passed through the body could reveal the internal body structure and organs. His discovery came to be known as x-ray and is today an essential tool in imaging various parts of the body to see if various systems and organs are healthy or not.

But while these historic medical discoveries were being made, the actual conditions of life in frontier America were often primitive and healthcare conditions were poor. A good example was the Civil War in which medicine often did more harm than good. For instance, disease far outranked actual combat in causing fatalities in that War, accounting for two of every three deaths recorded. Scurvy, dysentery, and diarrhea between them accounted for thousands of deaths on both sides of the War. In one year, 995 out of 1,000 soldiers in the Union Army came down with either diarrhea or dysentery or both. Sanitation was primitive or nonexistent.

It was in 1874 that osteopathic medicine had its beginnings in a frontier Missouri Town across from the Mississippi River, Kirksville. It was there that a courageous and determined country doctor, an MD, Dr. Andrew Taylor Still, became increasingly disenchanted with mainstream medicine. He had seen the

horrors of the Civil War and became increasingly bitter about the rudimentary medicine then being practiced and the largely ineffective drugs, after losing three of his own children. At that time, anesthesia, sterile surgery, and antiseptics although discovered, were largely unknown on the frontier and x-rays and antibiotics had yet to appear in common practice.

To combat what he felt were the inadequacies of the medicine being then practiced, Dr. Still developed his own unique system of medicine, based largely on the philosophy of Hippocrates. The keystone of Dr. Still's concept of medicine, was the unity of the body. And as the primary foundation of his system, he identified the musculoskeletal system - a concept which is receiving increasing acceptance today in mainstream medicine.

To make his new system more effective, he stressed the importance of palpation and touch as tools for diagnosing the ills of various patients. And he developed spinal manipulation as a less intrusive means of diagnosis and treatment. Today osteopathic medicine offers all of the accepted modalities and therapies of medical diagnosis and treatment - including x-ray, laboratory testing, physical and occupational therapy, to name but a few. But it also offers one important feature that is unique to osteopathy, the use of spinal manipulation at the option of the individual osteopathic physician, to use or not to use.

Today, osteopathic physicians continue to be on the cutting edge of medical technology. Like their allopathic counterparts, they can draw on the vast array of medical technology with their ears to listen closely to their patients, and with their eyes to see the patient as a human being and finally with their hands to diagnose and treat injury and illness.

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The Work You Will Do

If you want to understand what osteopaths do in carrying out their medical practices you must first understand the basic principles of osteopathy. First, osteopathic physicians believe that the body does heal itself, though it must constantly fight off physical, emotional, chemical and nutritional problems which can upset the body's functioning and well-being. DOs believe that they must educate patients about good health and what to do to lead a healthier and fuller life. The DO recognizes that medication and surgery can be useful elements of treatment, but also believes that spinal manipulation can often accomplish more than other treatments in restoring the body to health.

Once the DO has ruled out disease, infection, cancer and systemic disorders as causes of your illness (through blood and urine testing, x-ray and other diagnostic tools), he/she may decide to use manipulation. While many associate such manipulation as being primarily useful in assisting in treating such physical ailments as lower back pain, this tool can be far more useful than that in relieving a number of ailments including: asthma, carpal tunnel, menstrual pain, sinus disorders, migraine.

How Does It Work?

Twenty-six-year-old Renee visits her DO complaining of a sharp pain in her side. Her doctor asks her a series of questions about her pain, its intensity, onset, exact location, and duration. He also discusses her medical history. The physician then orders a series

of tests to learn if the pain is caused by disease (gallbladder, or appendix, or kidney). Tests revealed that it is not.

But Renee has revealed that she works in an office at a desk all day, and it occurs to him that Renee may be suffering from postural/mechanical strain. Based on his examination, interviews with the patient, and the negative test results, he decides to use manipulation to relieve motion restrictions and muscle strain caused by improper sitting and movement. It works. Renee feels 100 percent better, her pain gone, as if by a miracle. Her DO also recommends changes in the setup of her workstation to further lessen the chance for pain recurrences.

Michael's case is somewhat different. For some months now he has felt such symptoms as cough, congestion, fever and headache. Suspecting chronic sinusitis (inflammation of the nasal sinuses), the DO prescribes an antibiotic to combat the infection and the doctor also uses spinal manipulation to promote sinus drainage and relieve pain related to the infection. Michael's congestion is much better after just one osteopathic manipulation session and upon completion of his antibiotic treatment, just two weeks later, the symptoms disappear completely.

Aside from the emphasis on manipulation and their holistic concept of the body in relation to the entire working and social environment, DOs are very similar to MDs in their approach to medicine. For instance in examining a patient, they rely on palpation to obtain clues as to parts of the patient's anatomy or body which may be dysfunctional. They may also use a blood pressure cuff to check a patient's blood pressure, and a thermometer to get a temperature reading and a stethoscope to

check their heart beat, as well as the pulse beat in their hands which can be checked by the doctor manually.

The DO, as has already been noted, may order a lipid profile if he believes the patient to have too much cholesterol, and is thus a good candidate for a heart attack. He/she may also order a complete blood count to check blood cells, white and red, which among others may indicate the presence of an infection. Or the doctor may take a sample of the patient's tissue and have it analyzed under the microscope to see if the tissue is cancerous or not.

Osteopaths, like their MD colleagues, also take patient histories and take care of the reams of paperwork required for every action they have taken in treating a patient. In certain rural areas where there are no obstetricians, the DO may also deliver babies and order antibiotics for stubborn earaches, swollen or sore throats and other signs of infection.

Just how much patient contact the DO will have depends to a great extent on the specialization. The primary care specialist - the patient's first line of approach in case of an illness - will have contact almost from the start - after the patient is referred to the doctor for treatment, when the patient becomes ill or undergoes surgery and is hospitalized, and in the follow-up. If not actively caring for the patient, the DO may be acting as "gatekeeper" in referring patients with complex or unique problems to specialists for care and follow-up. But as the patient's primary contact, the primary care DO has to keep the patient apprized as to what the specialist has recommended and why, and to lend advice as to what to do in any given case.

Where You Will Work

Most osteopaths, estimated at 70 percent, practice chiefly in the states where osteopathic schools and hospitals are located: Michigan and Pennsylvania lead in numbers of DOs practicing there, followed by Ohio, Florida, New Jersey, and Texas. Other large concentrations of DOs are to be found in Illinois, Missouri, and California.

About two thirds of all osteopaths are in office based practices, including clinics and HMOs. About one fourth are hospital based. Others work for the federal government, in the Departments of Veterans Affairs, and in the Public Health Service and the Department of Health and Human Services.

Increasingly DOs are partners or salaried employees in group practices, for several reasons. As part of a clinic or group practice, the group can more easily purchase expensive medical equipment, and hire the staffs required to fill out the voluminous paperwork which doctors are increasingly being expected to handle before they can be paid by such third party payers as Medicaid, for the medically indigent, and Medicare, for those over 60. Through group practice the doctor as part of the group can count on having someone to cover for him/her when on vacation or taking time off for a personal or sick day.

The Professionals Speak

Specialist in Obstetrics who Practices in the Midwest

"In my last year of undergraduate college I won a National Science Foundation grant to attend Northwestern University, and for an assignment in my ethics course I had to write a paper on Ethics in Medicine. One student decided to study MDs, another chiropractors, others took pharmacists and nurses, and I didn't know what to take. Someone suggested osteopaths. I didn't know much about this branch of medicine, but I learned that there was a school across the Mississippi River in Kirksville, Missouri, which was a few hours away by car. So I called for an interview and when I arrived I saw that the others in the room all wore coats and ties and here I am in blue jeans. When I spoke to the dean of students, every time I asked him a question, he would ask me two. Finally, I told him that I was there for an interview for my ethics class report and he replied that he understood that I was there for admissions. So I finally understood that there had been a misunderstanding about my purpose in being there, and I was quickly dismissed but I did manage to get a tour of the school and learned of its history and met some of the students, and found it was interesting to learn more about the school and its program.

I returned to college, wrote my paper and graduated. Later, I was looking into medicine as a career and happened to get an application from the Kirksville College of Osteopathic Medicine. This was while I was in graduate school at Northwestern studying electron microscopy and insect biology. Meanwhile my National

Science Foundation grant was expiring and new grants were going mostly to experimental biology and cancer research. So I came home during Thanksgiving and came across the application from the Kirksville College. I can do this, I thought, filled out the application and returned to school. I returned to Kirksville for an interview, but of course this time with the purpose of being admitted to the school.

I liked Kirksville a lot. The people seemed genuinely interested in me, and I was impressed that osteopathy is a uniquely American form of medicine. Its main concepts are interesting since it is stressed that you work with your hands. In my studies, we looked at the musculoskeletal system as an entity unto itself and at its influence on health and disease. You might think of your body as a container and the inside contents as the inner workings of the body. There's a relationship to each other and how the container influences the contents. It's like architecture and engineering put together - the idea that structure and function are interrelated. I had never heard that before. Architecture of course is the musculoskeletal system, and engineering, the body functions or systems. Osteopathic manipulation is applied anatomy and physiology.

I took one extra year for an undergraduate fellowship to do a year of study of osteopathic manipulative medicine and was interested in working with my hands. My instructor was treating newborns with fabulous results in children with cerebral palsy, prematures, and children whose parents were told that their kids would be retarded.

Following graduation, we were asked to do a one year rotating internship - a little in various areas of medicine for one

postgraduate year.

In some states you could start a practice right after that. In others they require a residency completion before you could practice. I spent a year working in rural communities in Indiana, since this was the place to be to enhance my skills as a country doctor. I would do tonsilleotomies on a kitchen table and deliver babies at home - I would do everything. I wanted to make sure that I could handle fractures, perform a C-Section, deliver babies with forceps and do appendectomies, remove hemorrhoids and do tonsilleotomies. I wanted to do all this as preparation for being a family doctor. I also wanted to work in a place where there were not a lot of residents so that I could work directly with the attending physicians.

I learned how to insert a chest tube and could care for people in ICU (intensive care unit). I did all of this as part of my training at South Bend Osteopathic Hospital. I took my State Boards in Tennessee, one of the states which did not require a residency in order to practice. And I got reciprocal privileges to practice in Indiana and was able to moonlight in sports medicine caring for athletes with muscular strains and pulls; I was thus able to keep my manipulation skills.

I applied for a residency in Obstetrics-Gynecology and was accepted at Normandy Osteopathic Hospital in St. Louis, which is now a part of the Deaconess Medical System. I spent four years there, and then wanted to start a private practice specializing in Obstetrics-Gynecology in St. Louis. I felt that I could still continue to function as a general family practitioner, so I opened an office in St. Louis, working with another doctor from another osteopathic college, who had done a family practice residency also and felt

we could share patients. I took care of the pregnant patients and those with young children, and my partner took care of the older children. Both of us were kept busy.

Then I got married. My spouse is an MD who has a PhD in biological chemistry, is interested in clinical pharmacology research and started a clinical practice in pharmacology at the University of Chicago. I followed to Chicago about six months later and joined the staff at the Chicago College of Osteopathic Medicine, which was then in Hyde Park, near the University of Chicago. I also received an offer to teach in the osteopathic medicine department, and taught obstetrics as well. My practice expanded and I found myself doing more obstetrics and less manipulative medicine and teaching. I was the only obstetrician on the staff.

I then took a job at the Chicago Board of Health. I worked in a big project building for about a year and have never seen such sick people. I ran into drug addiction and TB on a scale which I had never before seen and this was quite educational.

I then obtained a position at a hospital on the city's North Side where two of the obstetricians on the staff had recently left. I shared on-call duty with several other doctors who all were MDs. Since many calls were from those who wanted osteopathic manipulation, I got all of these calls - from men, women and children.

Osteopathic manipulation is an excellent way to build a practice. I work on patients who have been to the Mayo clinic, headache clinics and still not found any relief. I can help pregnant women with backaches for instance and some who have asthma or who

may get very stiff during pregnancy and can be relaxed and loosened up through osteopathic manipulation.

My advice to those considering osteopathy as a career is that this is a unique concept of American medicine. Dr. Still, the founder was an MD, and his intention was not to start a separate discipline but to add to the healing profession with his theories and concepts of structure and function. His ideas are in keeping with the Hippocratic oath, which says that the physician should do no harm to a patient.

For those interested in osteopathic medicine, your hands are a diagnostic tool and you must put time into using your hands for palpatory diagnosis. There is information that you can get only by touching bodies before testing blood or taking an EKG. While my MD colleagues are good with the stethoscope and diagnostic laboratory tests, which I can utilize also, I can offer my skilled hands as well. This is a definite asset in doing a physical examination. Patients come in with stomach pains or cramps and a kink in the belly which may well come from some problems in the back.

If you want to go to osteopathic school and have a problem with people touching you or you touching people, then you should consider some other career, since you must allow others to palpate or touch you. In practice sessions, you will be touching them as well and must be willing to wear a t-shirt and shorts to allow for a full examination. If you are in any way doubtful of touching others or having them touch you, you might consider going into radiology, where this would not be a problem.

Admission to osteopathic school is tough these days. It's even tougher than getting into medical school. In recent years, some

DO graduates have been taken into MD residency programs. When I was a resident there were 11 osteopathic schools; today there are 19. Wherever you go, you have a bond of fellowship with other osteopathic school graduates. In a way you might consider the Kirksville school to be the Harvard of Osteopathic Medicine since it was the first established. The Philadelphia school is where those who want to become surgeons go, and Chicago is the school for training in internal medicine. Today in general, more DOs are being accepted into allopathic hospital staffs and medical societies, and one of our great gynecological-oncological surgeons is on the staff of a major hospital in the city."

Partner in a Suburban Group Practice

"I've been in practice now for nine years specializing in internal medicine. I am a graduate of the Chicago College of Osteopathic Medicine and did my residency at Cook County Hospital in the city. It was a straight MD residency and then I went into practice. I've been in several practices in Evanston, Lincolnwood and now primarily in Skokie. I am also on the medical staffs of St. Francis, Evanston and Glenbrook Hospitals.

As to osteopathic manipulation, I use it occasionally. Since I am an internist the majority of my practice involves such conditions as hypertension and diabetes. I have some patients who come in for manipulation, but it's more on a request basis. I don't bring it up.

Different DOs have different approaches to manipulation. Some are very heavily into manipulation. But basically I approach medicine like any other internist, DO or MD, and this is pretty much true of all DOs today.

We have the same training, take the same basic sciences in osteopathic college, the same clinical courses. But I do think that our approach is more holistic. There's more of an emphasis on being open-minded and accepting of alternative treatments and a willingness to accept the possibilities of some other treatments working to the patient's benefit. I still believe in the scientific approach - that things need to be proven in trials - double blind, placebo controlled trials - before they are effective. But if a patient asks me if it would bother me if they saw a chiropractor, that doesn't bother me in the least - or if they want to follow a nutritional approach as opposed to trying medicine, that's okay. Ideally, I emphasize exercise, weight control and dietary control. That really gets to the root of most of the problems that I see.

A big portion of my practice is devoted to patients with coronary disease, hypertension, diabetes and it runs pretty much the gamut of what middle aged and the elderly patients have.

I just changed my group and most of my patients followed me. Part of my agreement on leaving was that I could have my patients' charts and the ability to follow-up on my patients.

Our osteopathic training is in one respect superior because of the additional training in spinal manipulation. To learn this we have a class several times a week for about two years. But while you are in osteopathic training, you learn just about everything in medicine. I feel that DOs try harder to listen to their patients. I've gotten such good feedback from several patients.

When I started my staff training at an MD institution, I was concerned if it would be okay. I was there for several years and I got the 'Attending of the Year,' from the house staff. Currently,

the head of OB-Gyne is a DO, the head neurologist is a DO and one of the best emergency room doctors is a DO.

As internists there are probably three or four DOs, and three or four OB-Gynes. I would say that probably a total of 15 of the staff doctors are DOs out of about 200. Since I have been out I have been associated with the medical staffs of two institutions, and there probably is about that same ratio - approximately five percent osteopaths - currently on staff at these other hospitals.

In the short time that I have been practicing the number of DOs on staff has grown. When I first graduated there may have been only one or two others besides myself on staff. Medical societies welcome me and I belong to the local county and state societies as well as the AMA. So the difference between the two branches has been lessened and blurred so that they are practically the same.

When I became interested in medical school, I had applied to several allopathic schools as well as osteopathic colleges. But my first interview was at Chicago Osteopathic and just from that, from the school, from the Dean of Admission, from several professors there, I found that they were interested in me as a person, not just as a physician. I received my letter of acceptance and decided on the spot that was where I wanted to go. So I passed on interviews at the University of Illinois and Loyola.

I stay in touch with a number of my classmates. There were a hundred of us that started, and there were 92 that graduated. And those that I've stayed in touch with are doing fine. One chap is the only pediatric orthopedic doctor in Northern Michigan, and another friend is a neurosurgeon in Florida and he is doing fine.

At Evanston hospital there are a couple of OB-Gyne attending to the needs of several high risk children.

As to workload it's about the same as for other physicians, more than 60 hours a week. Actual time spent in seeing patients is about 30-35 hours per week. But I also have to check on my patients at the hospitals. And I have to follow up on them after they are released. I would say I put in 60 hours a week easily. And every other Saturday half a day. In my present group we take turns being on call - so every seven weeks I am on call, and that's one advantage of working in a group. I handle all phone calls and check on all of the hospitalized patients to make sure they are okay.

A typical day will depend on how many patients I have in the hospital but it's rare that I don't have somebody in. For instance, this morning I was at the hospital by about 7:30 a.m. and saw my two patients at St. Francis. I presently have no patients at Evanston, but if I did, I'd have to see them and that holds true of Glenbrook as well. Sometimes I start at 8 a.m. and am working toward earlier hours - so I see patients from about 8 to 10 a.m. and at noon everything shuts down for an hour for lunch. Then I'll typically see patients from about 1 to 5 p.m.

But we also are open Saturdays for a half day. And when I get home, I do on the average of a half hour to an hour of paperwork or call backs. I see patients about every 15 or 30 minutes in the office and when patients call during the day, I get about 15 to 25 messages. I can get a number of those call backs done during the day and then must take the rest home. Last night at home, I called four people to talk over either lab results or general questions.

You try to keep up with reading. You go to seminars at the hospital or grand rounds - which is once a week and is usually a one hour session with a speaker who discusses current topics - such as depression or latest treatments for coronary artery disease or sleep apnea.

I don't have too many problems with patients or pressures of work. Maybe it's just a matter of time, but I've learned to become very comfortable with the patients and mapping out a plan together. That's my style. I don't dictate what I am going to have them do. I tell them what I think about the problems and give them options and insights. For instance in the case of a patient with high blood pressure, either the patient is going to make changes in lifestyle, lose weight and watch salt intake, or sometimes the patient will not, which is probably 80 percent of the time. In that case, I strongly press for them to take a blood pressure medicine, but I can't force them to do this.

Today in medicine, the patient comes in to explain problems, symptoms, or whatever, and then the doctor asks for more details. You come up with a working theory as to the diagnosis and try to prove it as best as you can. I try to avoid using expensive high tech treatments. Let's say that you come in with a headache - this can encompass anything from a simple muscle tension headache to a brain tumor. So should I get a CAT scan on every patient that comes in with a headache? You're not really practicing good medicine if you are doing that.

So you do an examination, ask questions, look at their eyes, do a neurologic exam and try to determine what kind of headache this person has and what's causing it. I then review this with the

patient and either start the medication or suggest exercises, stress reduction, and other therapies.

No, I have no problem in my dealings with patients. But where I do find pressure is in the business aspect of the practice. And the frustration of dealing with managed care regulations. I work with a number of HMOs, and there are several models or kinds of HMOs. The most controlled type is an HMO where the patient is assigned a primary care doctor who acts as gatekeeper. As such you are supposed to prevent frivolous tests and procedures from being ordered or done, but on the other hand the gatekeeper is expected to deliver good medical care, treat the patient well, help the person live longer and still follow the HMO guidelines. And for this the primary care doctor is awarded the grand sum of \$7 to \$10 per patient; it gets very frustrating. If you dealt only with one HMO it would not be so bad. But in this group we work with six HMOs, each with its own requirements and regulations. They are somewhat similar, but still there are differences. There is also the service model which is not as restrictive, but still requires referral approval to see people not connected with the HMO.

Then you have a PPO which is less restrictive. We have one staff person who devotes the entire day to dealing with referrals and with insurance companies. To refer to a specialist, you must get a written referral and approval. Simple things may not require approval, but you have to know these requirements. If a patient calls you in the middle of the night, after I talk to the caller, my next question is, 'Let's go over your insurance. Have you HMO Illinois? Which hospital are you affiliated with? Where should I have you go?' If I select a hospital that the patient is not affiliated with, the patient may have to pay for the visit to the Emergency Room.

Such red tape makes you angry and makes the practice of medicine seem uncaring. You deal with it and work with it as best you can. But you certainly don't have to embrace it.

I would think that someone who's interested in becoming a DO would have to be aware that it is a more holistic approach and that it encompasses more use of manipulation.

To succeed, you must first of all like people. You have to be able to talk to people and to work with people to make it as a physician. You have to have empathy and patience. You have to listen to an awful lot to be able to pull out some ideas which may help the person.

Sure you run into some difficult patients. But you learn in time to accept this and that you will not be able to please everyone.

You don't have to be an absolute genius to become a physician. But you must know your limitations, when a patient's needs are beyond the scope of your ability to handle, for example. You must be able to sit and listen to patients, and need to involve the patient in the care. Some older patients cannot accept this. But mostly I sit down and try to point out the options. If they insist that they want you to make the decisions, then I will say, 'Look this is what I want you to do.'

My advice to those who are really interested in this field is they should try to go to a doctor's office and just spend some time with a professional. I have written a number of letters for students interested in the career, but I insist that these college students come in and work with me for a while. I have them come into the room and introduce them to a patient. I want them to see what I do before I will write them a letter. I had one

student who persisted in coming in to see and work with me, and although he was not admitted to school the first year he tried, he persisted and was accepted the second time he tried. He is now practicing in Iowa and doing just fine."

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Personal Qualifications

When measuring your suitability for the career you might take a personal inventory. Be honest with yourself. Are you up to meeting the very high academic and personal demands of the work? Many physicians work 60 hour weeks on average, and the medical school curriculum is very tough, especially for the first two basic science years. For every hour of class or lab work that you put in, you should reserve at least that much and perhaps twice that amount in study -either at school or at home.

Ask yourself if you are really dedicated to serving the public. Are you willing to work the long work weeks and the interference with your own personal life and needs in order to take care of patients who may call you up in the middle of the night?

Can you withstand the ravages of patients who are dying of some incurable disease? Patients can often be unreasonable and very demanding on their physicians especially when they are in pain or not responding to treatment as expected. In such cases, you require considerable patience and composure to carry on your work.

Above all you must feel comfortable with people. Do you enjoy meeting strangers, people in all walks of life, whose beliefs, customs and language may differ from your own?

How are you under pressure? You may have several patients all of whom become sick at the same time, and it may be a strain to know which to handle first.

If you believe that you would be able to manage these types of situations then perhaps you should think further of pursuing a medical career. But if you have doubts, you might be better off channeling your interests into other careers which might not involve so much responsibility or pressure, such as physician assistant, medical technologist or physical, occupational or speech therapist.

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Attractive Features

The doors have never been open as widely to osteopathic physicians and your chances of joining the staff of the hospital of your choice have never been so good. And the outlook is good for continued improvement in the healthcare climate which will see more and more hospitals opening up to osteopathic physicians and more residency programs admitting graduates of osteopathic schools.

The earnings of osteopathic physicians are excellent, averaging about \$185,000 annually for all physicians. The average for specialists, such as anesthesiologists and obstetricians is over \$200,000.

While many disagree about the future of some of the specialties in medicine, both allopathic and osteopathic, there is widespread agreement that physicians in primary care can find many excellent opportunities in medicine, as well as many specialists, especially cardiologists, urologists, gastroenterologists, and ophthalmologists.

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Unattractive Features

On the minus side, osteopathic physicians do work long and irregular hours. About half of all full-timers average 60 hours or more. Patients become ill at all times of the day or night, and you may be called on to see a patient in the evening who has taken a turn for the worse, or who has been admitted to the hospital and is asking to see you - this despite the fact that you may have made other plans. In medicine, unless you can restrict your practice to certain days, you are literally on call, almost any time of the day or night.

We have already referred to the fact that in certain areas, where allopathic medicine is very strong, and in certain metropolitan areas you may have trouble gaining staff privileges at certain hospitals or qualifying for certain residency programs.

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Education and Training

Education for osteopathic medicine is not easy. It can require a total of at least 11 years of college and post college work to complete: four years of undergraduate (college) work, four years of osteopathic college, and at least three years of residency (specialty training), often more. For some residencies, such as neurosurgery, residency can take longer, up to eight years.

But first you must be admitted to an osteopathic school and even that is not easy - only about 20 percent will be admitted. And to be considered at all, you must complete college work in physics, biology, mathematics, English and organic chemistry. The exact number of hours and courses that you should take varies from school to school, so it is essential that you check with the individual schools in which you are interested for their specific requirements. You should also take courses in the humanities and the social sciences, since getting along with patients is as much an art as a science.

To be admitted to osteopathic school, you must have completed at the very least three years of college, but the overwhelming majority of applicants have at least their bachelor's degrees and many have advanced degrees as well. There are currently 19 osteopathic colleges in the United States (for a list of the accredited schools, write to the Central Application Service of the American Association of Colleges of Osteopathic Medicine).

Acceptance is very competitive and is based on a number of factors. These include your grades on the MCAT (very important), transcripts of your college grades, and letters of

recommendation. In addition, most schools require an interview with the admissions officer and many ask that you submit a short essay on why you wish to be admitted to osteopathic college. Schools are also looking for volunteer service in hospitals or various public or private health agencies, and they consider your character, personality, leadership qualities and participation in extracurricular activities.

Once you are admitted to osteopathic college, you will spend most of the first two years, known as the basic science years, in laboratories and classrooms studying such courses as anatomy, biochemistry, physiology, pharmacology, psychology, microbiology, pathology, medical ethics and laws dealing with medicine. Almost universally students agree that these two years are very tough, and you are expected to know and assimilate a great deal of material, most of which may seem at the time to have little or no bearing to what you do in medicine.

You will also have some patient contact, in almost all schools, learning how to take patient histories, and how to examine patients, and what symptoms to look for in the course of your examination.

The second two years of osteopathic school, which most students find much more to their liking, are called the clinical years, in which you work with patients under the eyes of experienced physicians to learn about acute disorders (often very serious, but usually temporary, such as a heart attack), chronic disorders (asthma, arthritis, diabetes, and so forth), and preventive and rehabilitative care. Through rotations in various areas of medicine, lasting anywhere from a few weeks to a few months and longer, you are exposed to the various medical fields,

including internal medicine, family practice, obstetrics and gynecology, pediatrics, psychiatry and surgery. And you gain experience in the diagnosis and treatment of disease as well by accompanying the attending physician on hospital rounds, observing the way they treat patients and studying the symptoms and diagnoses of each patient.

Following medical school, you will be expected to serve a 12 month revolving internship, where again you are exposed to the various areas of medical practice and treatment. You are then eligible to take an examination of the National Board of Osteopathic Examiners. Following your internship, you may wish to pursue a residency program for training in a specialty area, or you may wish to apply for your license to practice as a DO.

To be licensed as a DO, you must in all states, as well as the District of Columbia, be a graduate of an accredited school of osteopathy, pass a licensing examination and have completed one to seven years of graduate education, an internship and a residency for DOs.

If you are seeking Board Certification in any of the 34 medical specialties and subspecialties, it could take you as much as three to four additional years of training to qualify. A final examination immediately following residency or after one or two years of practice in that specialty is required for Board Certification in your specialty by the American Osteopathic Association. There are presently about 34 specialty boards. Write to the American Osteopathic Association for a listing of the various specialties and the specific requirements of each for Board Certification. Certification means simply that you are now qualified to practice

as a specialist, for example, an obstetrician, radiologist, or internist.

Once you have passed all of the hurdles and are in practice your education is not ever complete, for medicine is one field in which you are expected to keep up to date so you must meet certain requirements in the areas of Continuing Education. The American Osteopathic Association requires that its members earn a specified number of continuing medical education credits every three years in order to maintain membership. Credits can be achieved by reading certain professional journals and taking tests to make certain that you have extracted certain basic information from your reading, or by attending various medical meetings and symposia.

To teach or do research, you may need a master's degree or PhD, in such fields as biochemistry or microbiology.

An osteopathic education is expensive. The annual cost of attending osteopathic college averages almost \$23,000 for private school and almost \$9,000 in public (state supported) schools. To pay for this, you may try to work out a package of loans, scholarships or grants, and work/study programs with the school's financial officer. These days it is not at all unusual for students to finish osteopathic school \$80,000 or more in debt. And while this may seem like a lot, and it is, most of these loans are not payable until you begin a residency, when you will actually begin earning a minimal salary while you are in training.

The details of what loans and scholarships are available, interest payable and when it is payable, and qualifications for receiving such assistance vary quite a bit and it is strongly suggested that

you get together with the financial officer of all schools which you visit to see which has the most attractive financial package.

Listed below are accredited Osteopath Colleges & Universities

- [A.T. Stills University](#)
- [Arizona College of Osteopathic Medicine](#)
- [Chicago College of Osteopathic Medicine](#)
- [Des Moines University](#)
- [Philadelphia College of Osteopathic Medicine - Georgia Campus](#)
- [Kansas City University of Medicine & Biosciences](#)
- [Lake Erie College of Osteopathic Medicine](#)
- [Lincoln Memorial University](#)
- [Michigan State University](#)
- [Nova Southeastern University](#)
- [Oklahoma State University Center for Health Sciences](#)
- [Ohio University College of Osteopathic Medicine](#)
- [Pacific Northwest University of Health Sciences](#)
- [Rocky Vista University College of Osteopathic Medicine](#)
- [University of New England](#)
- [University of North Texas Health Science Center at Fort Worth](#)
- [West Virginia School of Osteopathic Medicine](#)

Earnings

The average annual earnings of all physicians are about \$185,000. The average annual income of salaried physicians is almost \$150,000. Salaried physicians don't own part of their practice and work for others. Self-employed physicians, on the other hand, are those who own part or all of their practice.

Although the earnings of salaried physicians are generally lower than those of self-employed doctors, those on salary enjoy greater economic security and predictability, and of course do not have to worry about the considerable investment in time and money involved in establishing a practice. Salaried physicians are employed by hospitals, group practices and managed care plans. Group practices pay physicians somewhat more than hospitals or managed care plans.

Physicians in medical specialties report average earnings as follows:

- Internal Medicine, \$135,000
- Family Practitioners, \$130,000
- General Surgery, \$200,000
- Oncology, \$165,000
- Anesthesiology, \$195,000
- Pathology, \$170,000
- Radiologists, \$200,000
- Cardiologists, \$210,000
- Anesthesiologists, \$205,000
- Obstetrics-gynecology, \$200,000

Bear in mind that earnings vary according to number of years in practice, geographic regions, hours worked and kind of practice.

Medical residents' salaries range from \$30,000 for those in their first year of residency to \$40,000 for those in their eighth year.

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Opportunities

Most medical experts see a good future for physician employment which is expected to grow faster than the average for all occupations. Breakthroughs in medical technology enable physicians to do more tests, perform more procedures and to treat many conditions previously thought of as untreatable.

Job prospects are particularly good for primary care physicians such as family practice doctors, pediatricians and internists. Because of efforts by insurance companies and the government to control health costs and increased dependence on guidelines which limit hospital utilization and treatment, a lower demand is foreseen for specialists.

Some healthcare analysts predict too many physicians in certain specialties; others say that the problem is one of "maldistribution." For instance, there are many shortages in rural areas and communities of fewer than 50,000 population for several reasons: physicians in general find these areas offer lower earnings potential and that they are isolated from medical associates. This is not as true of osteopathic medicine, as much as of the MD branch, since more osteopaths practice in smaller, rural communities than their MD counterparts. Also, fewer osteopathic physicians tend to specialize, another factor which would work toward their benefit - since generalists or primary care physicians will be in greater demand than many specialists.

Getting Started

About 65 percent of all new physicians choose to take jobs working for others, such as hospitals, HMOs and other managed care plans, and clinics. There are many reasons for this. For one, many medical students are leaving school, osteopathic or medical, with debts approaching as much as \$100,000 and either lack the funds required to begin their own practice or feel more secure working for others. After all starting a solo practice involves a risk; there is no assurance that you will survive financially during the first few years; you may not be able to show any income for your investment.

There are other reasons such as the fact that many new physicians find it more comforting to be able to communicate with others either as part of a group or as a salaried physician on a hospital staff, plus the fact that you usually can find someone to cover your calls if you want to take a vacation or a day off. Also salaried physicians usually have good benefits: paid vacations, holidays, and sick days, as well as healthcare coverage, participation in pension programs, and many others.

But there is a price to pay. For one there is your loss of independence. Your hours, when you can practice and many other options are regulated by the group, and this may not be so convenient for you. Second, the solo practitioner in general has the opportunity for higher earnings than the salaried doctor.

Although taking an employed position is the approach followed by most new doctors, it is a fact that in a few years, the situation

changes quite drastically, from over 65 percent employed to less than 35 percent after six to eight years of practice. These are overall percentages, and this figure for employed physicians varies quite a bit by specialty. Almost all pathologists with two or fewer years of practice are employed and over 80 percent of radiologists with similar experience. Only about 40 percent of psychiatrists and 50 percent of general surgeons have not gone into their own private practice after two years as doctors.

Getting started will depend largely on where you do your residency. During residency many residents receive offers from attending staff who are looking for junior partners, or from the hospitals where they do their residency.

There is also the possibility of finding positions at local, state, national and specialty society meetings, or by reading medical journals published by these organizations. Often such journals run ads for physicians wanted or for new graduates seeking positions.

In seeking a new job, it is suggested that you by all means check out any openings by traveling to the area where the opening exists and discussing the job with the person or physician in charge. Besides professional journals and medical societies, there are many medical employment agencies that can help you find a job, and many medical schools may list opportunities available for those who have completed their residencies.

Getting ahead is usually an option that may be open to you with experience and proof of your ability to handle a job with more patients or responsibility or by joining a group practice in a higher income suburb than where you are currently practicing or employed.

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Associations

Listed below are some prominent trade associations:

- [American Association of Colleges of Osteopathic Medicine](#)
- [American Osteopathic Foundation](#)
- [American Academy of Osteopathy](#)

Listed below are some trade periodicals

- [Journal of the American Osteopathic Association](#)
- [Modern Healthcare](#)
- [AAO Journal](#)
- [Student Doctor](#)

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